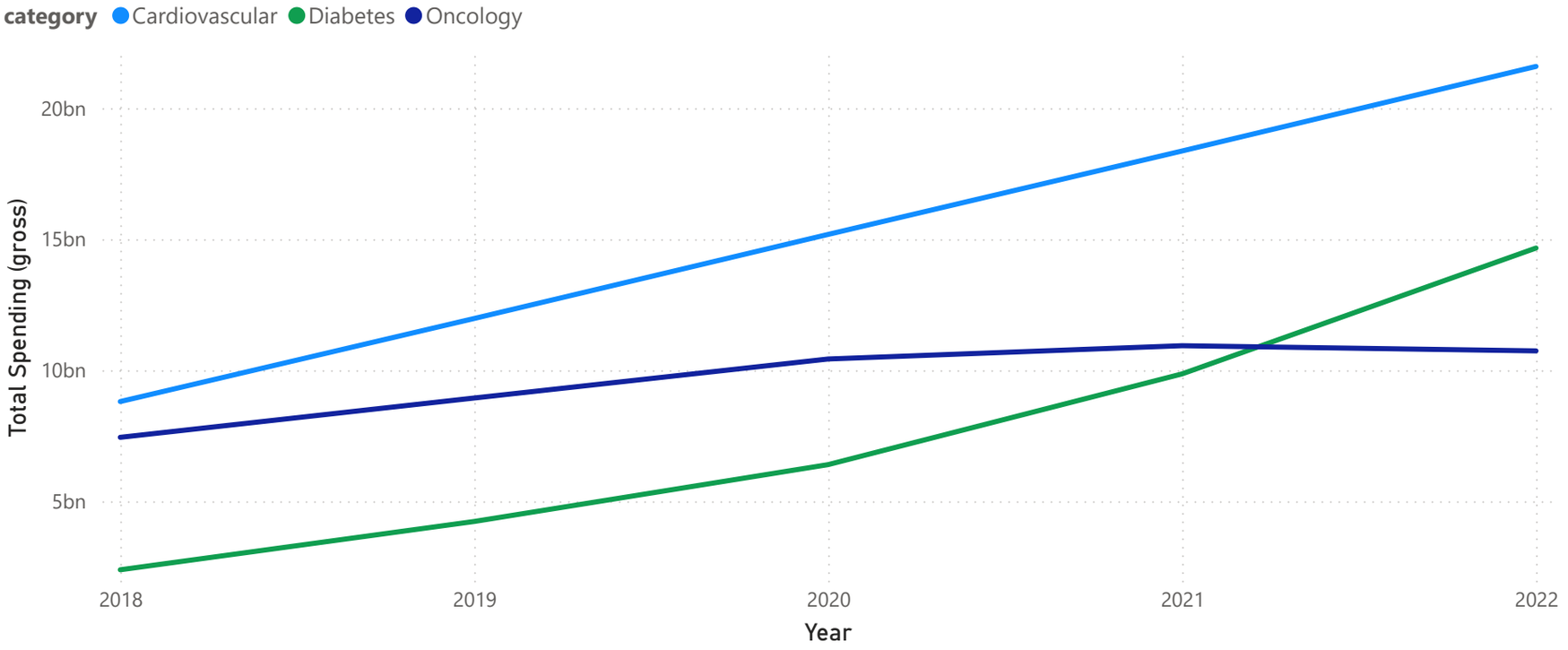
18.63bn
Spending 2018

46.99bn
Spending 2022

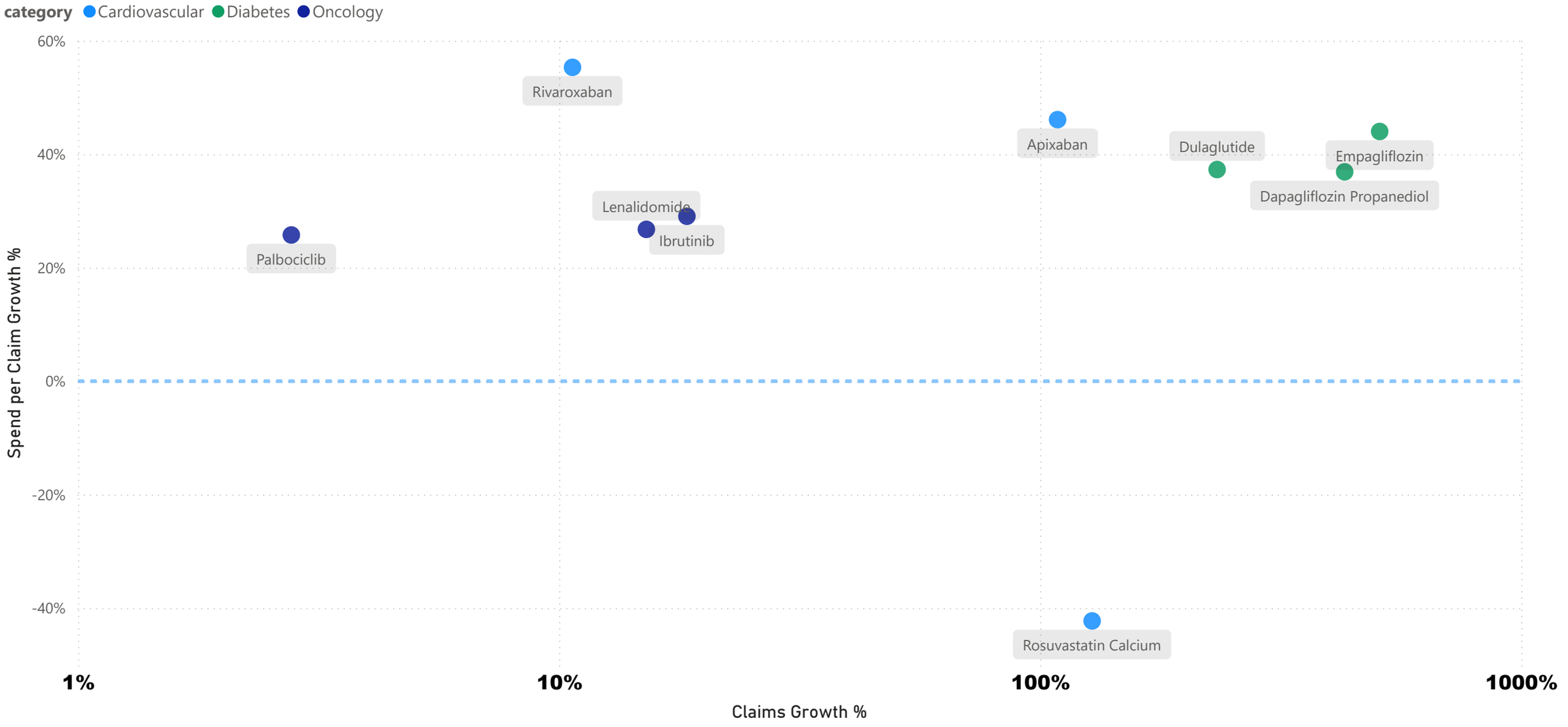
152.2%
Spending Growth %

Spending on 9 drugs rose 152% in four years



9 selected drugs (3 per category), not whole categories. Gross spending, before manufacturer rebates. Source: CMS Medicare Part d .

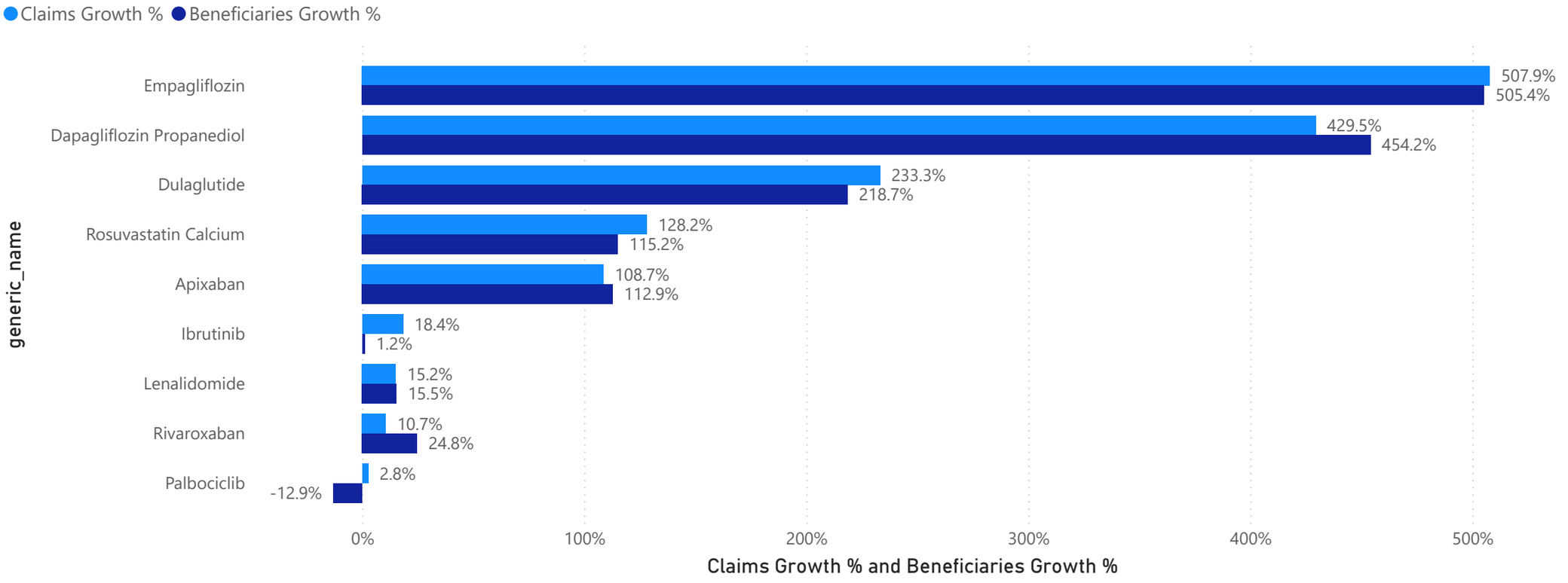
Spend per claim rose for 8 of 9 drugs; only Rosuvastatin fell



X axis is log scale. Growth 2018 to 2022. Spend per claim = Total spending / Total claims .

generic_name	category	Spending Growth %	Claims Growth %	Gap (pts)	Spend per Claim Growth %	Dollar Change	Driver Label
Apixaban	Cardiovascular	204.9%	108.7%	96.20	46.1%	$10.23bn	Spend per claim outgrew claims
Empagliflozin	Diabetes	775.5%	507.9%	267.57	44.0%	$5.18bn	Spend per claim outgrew claims
Dulaglutide	Diabetes	357.6%	233.3%	124.32	37.3%	$4.86bn	Spend per claim outgrew claims
Rivaroxaban	Cardiovascular	71.9%	10.7%	61.22	55.3%	$2.41bn	Spend per claim outgrew claims
Dapagliflozin Propanediol	Diabetes	625.0%	429.5%	195.41	36.9%	$2.23bn	Spend per claim outgrew claims
Lenalidomide	Oncology	46.0%	15.2%	30.82	26.8%	$1.87bn	Spend per claim outgrew claims
Ibrutinib	Oncology	52.9%	18.4%	34.43	29.1%	$0.99bn	Spend per claim outgrew claims
Palbociclib	Oncology	29.3%	2.8%	26.48	25.8%	$0.44bn	Spend per claim outgrew claims
Rosuvastatin Calcium	Cardiovascular	31.8%	128.2%	-96.46	-42.3%	$0.14bn	Claims outgrew spend
Total		**152.2%**	**117.9%**	**34.32**	**15.8%**	**$28.36bn**	**Spend per claim outgrew claims**

Claims and patients mostly grew together; Palbociclib lost patients as claims rose.



Findings

- Spending on the 9 drugs rose 152% ($18.63B to $46.99B) while claims rose 118%.
- For 8 of 9 drugs, spending grew faster than claims. Spend per claim rose 26% to 55%.
- Rosuvastatin is the exception: claims rose 128% while spend per claim fell 42%.
- Percent and dollar rankings differ. Empagliflozin grew most in percent (+775%), but Apixaban added the most dollars (+$10.2B).
- Oncology claims rose only 3% to 18%, while its spending rose 29% to 53%.
- Portfolio spend per claim rose only 15.8%, likely because Rosuvastatin, about a third of all claims, fell 42%.

Limitations

- Spending is gross, before manufacturer rebates, so net cost is lower.
- Only 2018 and 2022 are compared per drug.
- "Spend per claim outgrew claims" does not separate price from dose, days supply or patient mix.
- Only 9 drugs, 3 per category, not whole categories.
- Rosuvastatin is the generic-labelled row only. Branded Crestor is not in the data, so brand-to-generic shifts can't be ruled out.

Source: CMS Medicare Part D, 2018-2022. 9 selected drugs.